LABORATORY REPORT

Redcliffe labs

Patient Name	**: Ms Ch Padma Vati**		
DOB/Age/Gender	: 61 Y/Female	Bill Date	: Mar 20, 2024, 05:13 PM
Patient ID / UHID	: 7653250/RCL6812930	Sample Collected	: Mar 23, 2024, 07:46 AM
Referred By	: Dr.	Sample Received	: Mar 23, 2024, 02:06 PM
Sample Type	: Whole blood EDTA	Report Date	: Mar 23, 2024, 03:33 PM
Barcode No	: HY333777	Report Status	: Final Report

NABL M(EL)T LABS
NABL-M(EL)T-00728

HEMATOLOGY REPORT

Smart Plus Full Body Checkup With Vitamin Screening

Complete Blood Count (CBC)

Test Description	Value(s)	Unit(s)	Reference Range
RBC Parameters			
Hemoglobin Method : colorimetric	12.5	g/dL	12.0 - 15.0
RBC Count Method : Electrical impedance	4.5	10^6/µl	3.8 - 4.8
PCV Method : Calculated	38.2	%	36 - 46
MCV Method : Calculated	84.5	fl	83 - 101
MCH Method : Calculated	27.6	pg	27 - 32
MCHC Method : Calculated	32.6	g/dL	31.5 - 34.5
RDW (CV) * Method : Calculated	**18.8**	%	11.6 - 14.0
RDW-SD * Method : Calculated	**50.3**	fl	35.1 - 43.9
WBC Parameters			
TLC Method : Electrical impedance and microscopy	9.3	10^3/µl	4 - 10
Differential Leucocyte Count			
Neutrophils	55.9	%	40-80
Lymphocytes	35.6	%	20-40
Monocytes	3.9	%	2-10
Eosinophils	4.3	%	1-6
Basophils	0.3	%	<2
Absolute Leukocyte Counts **Method : Calculated**			
Neutrophils.	5.2	10^3/µl	2 - 7
Lymphocytes.	**3.31**	10^3/µl	1 - 3
Monocytes.	0.36	10^3/µl	0.2 - 1.0
Eosinophils.	0.4	10^3/µl	0.02 - 0.5
Basophils.	0.03	10^3/µl	0.02 - 0.5
Platelet Parameters			
Platelet Count Method : Electrical impedance and microscopy	228	10^3/µl	150 - 410
Mean Platelet Volume (MPV) *	11.4	fL	9.3 - 12.1

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NABL M(EL)T LABS
NABL-M(EL)T-00728

Test Description	Value(s)	Unit(s)	Reference Range
Method : Calculated			
PCT *	0.3	%	0.17 - 0.32
Method : Calculated			
PDW *	15.4	fL	8.3 - 25.0
Method : Calculated			
P-LCR *	38.4	%	18 - 50
Method : Calculated			
P-LCC *	88	%	44 - 140
Method : Calculated			
Mentzer Index *	18.78	%	-
Method : Calculated			

Interpretation:
CBC provides information about red cells, white cells and platelets. Results are useful in the diagnosis of anemia, infections, leukemias, clotting disorders and many other medical conditions.

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Test Description	Value(s)	Unit(s)	Reference Range

HEMATOLOGY REPORT

Smart Plus Full Body Checkup With Vitamin Screening

Erythrocyte Sedimentation Rate (ESR)

ESR - Erythrocyte Sedimentation Rate	18	mm/hr	0 - 20

Method : MODIFIED WESTERGREN

Interpretation:

ESR is also known as Erythrocyte Sedimentation Rate. An ESR test is used to assess inflammation in the body. Many conditions can cause an abnormal ESR, so an ESR test is typically used with other tests to diagnose and monitor different diseases. An elevated ESR may occur in inflammatory conditions including infection, rheumatoid arthritis ,systemic vasculitis, anemia, multiple myeloma , etc. Low levels are typically seen in congestive heart failure, polycythemia ,sickle cell anemia, hypo fibrinogenemia , etc.

AGE	MALE	FEMALE
1 DAY	0-2	0-2
2 - 7 DAYS	0-4	0-4
8 - 14 DAYS	0-17	0-17
15 DAYS - 17 YEARS	0-20	0-20
18 - 50 YEARS	0-10	0-12
51- 60 YEARS	0-12	0-19
61 - 70 YEARS	0-14	0-20
71 - 100 YEARS	0-30	0-35

Reference- Dacie and lewis practical hematology

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Referred By	: Dr.	Sample Received	: Mar 23, 2024, 02:06 PM
Sample Type	: Whole blood EDTA	Report Date	: Mar 23, 2024, 03:43 PM
Barcode No	: HY333777	Report Status	: Final Report

NABL M(EL)T LABS NABL-M(EL)T-00728

Test Description	Value(s)	Unit(s)	Reference Range

HEMATOLOGY REPORT

Smart Plus Full Body Checkup With Vitamin Screening

HbA1C (Glycosylated Haemoglobin)

Glycosylated Hemoglobin (HbA1c) Method : HPLC	**6.1**	%	< 5.7
Estimated Average Glucose *	128.37	mg/dL	Refer Table Below

Interpretation:

Interpretation For HbA1c% As per American Diabetes Association (ADA)

Reference Group	HbA1c in %
Non diabetic adults >=18 years	<5.7
At risk (Prediabetes)	5.7 - 6.4
Diagnosing Diabetes	>= 6.5
Therapeutic goals for glycemic control	Age > 19 years Goal of therapy: < 7.0 Age < 19 years Goal of therapy: <7.5

Note:

1. Since HbA1c reflects long term fluctuations in the blood glucose concentration, a diabetic patient who is recently under good control may still have a high concentration of HbA1c. Converse is true for a diabetic previously under good control but now poorly controlled.
2. Target goals of < 7.0 % may be beneficial in patients with short duration of diabetes, long life expectancy and no significant cardiovascular disease. In patients with significant complications of diabetes, limited life expectancy or extensive co-morbid conditions, targeting a goal of < 7.0 % may not be appropriate.

Comments :

HbA1c provides an index of average blood glucose levels over the past 8 - 12 weeks and is a much better indicator of long term glycemic control as compared to blood and urinary glucose determinations ADA criteria for correlation between HbA1c & Mean plasma glucose levels.

HbA1c(%)	Mean Plasma Glucose (mg/dL)	HbA1c(%)	Mean Plasma Glucose (mg/dL)
6	126	12	298
8	183	14	355
10	240	16	413

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Barcode No	: HY333777	Report Status	: Final Report

NABL M(EL)T LABS
NABL-M(EL)T-00728

Test Description	Value(s)	Unit(s)	Reference Range

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Referred By	: Dr.	Sample Received	: Mar 23, 2024, 02:06 PM
Sample Type	: FLUORIDE F	Report Date	: Mar 23, 2024, 03:35 PM
Barcode No	: ZC077780	Report Status	: Final Report

NABL M(EL)T LABS NABL-M(EL)T-00728

Test Description	Value(s)	Unit(s)	Reference Range

BIOCHEMISTRY REPORT

Smart Plus Full Body Checkup With Vitamin Screening

Glucose Fasting (BSF)

Glucose Fasting Method : Hexokinase	**126**	mg/dL	70 - 100

Interpretation:

Status	Fasting plasma glucose in mg/dL
Normal	<100
Impaired fasting glucose	100 - 125
Diabetes	=>126

Reference : American Diabetes Association

Comment :
Blood glucose determinations in commonly used as an aid in the diagnosis and treatment of diabetes. Elevated glucose levels (hyperglycemia) may also occur with pancreatic neoplasm, hyperthyroidism, and adrenal cortical hyper function as well as other disorders. Decreased glucose levels (hypoglycemia) may result from excessive insulin therapy insulinoma, or various liver diseases.

Note
1.The diagnosis of Diabetes requires a fasting plasma glucose of > or = 126 mg/dL or a random / 2 hour plasma glucose value of > or = 200 mg/dL with symptoms of diabetes mellitus.
2.Very high glucose levels (>450 mg/dL in adults) may result in Diabetic Ketoacidosis.

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Barcode No	: ZC077781	Report Status	: Final Report

NABL M(EL)T LABS NABL-M(EL)T-00728

Test Description	Value(s)	Unit(s)	Reference Range
BIOCHEMISTRY REPORT			
Smart Plus Full Body Checkup With Vitamin Screening			
Liver Function Test (LFT)			
Bilirubin Total Method : Photometric	0.4	mg/dL	0.2 - 1.2
Bilirubin Direct * Method : Diazo Reaction	0.2	mg/dL	0.0 - 0.5
Bilirubin Indirect * Method : Calculation (T Bil - D Bil)	0.2	mg/dL	0.1 - 1.0
SGOT/AST Method : IFCC without P5P	18	U/L	5 - 34
SGPT/ALT Method : IFCC without P5P	15	U/L	0 to 55
SGOT/SGPT Ratio *	1.2	-	-
Alkaline Phosphatase Method : IFCC	75	U/L	40 - 150
Total Protein Method : Biuret	7.6	g/dL	6.4 - 8.3
Albumin Method : BCG	4.4	gm/dL	3.8 - 5.0
Globulin * Method : Calculation (T.P - Albumin)	3.2	g/dL	2.3 - 3.5
Albumin :Globulin Ratio * Method : Calculation (Albumin/Globulin)	1.38	-	1.0 - 2.1
Gamma Glutamyl Transferase (GGT) * Method : Photometric	16	U/L	9 to 36

Interpretation:

The liver filters and processes blood as it circulates through the body. It metabolizes nutrients, detoxifies harmful substances, makes blood clotting proteins, and performs many other vital functions. The cells in the liver contain proteins called enzymes that drive these chemical reactions. When liver cells are damaged or destroyed, the enzymes in the cells leak out into the blood, where they can be measured by blood tests Liver tests check the blood for two main liver enzymes. Aspartate aminotransferase (AST),SGOT: The AST enzyme is also found in muscles and many other tissues besides the liver. Alanine aminotransferase (ALT), SGPT: ALT is almost exclusively found in the liver. If ALT and AST are found together in elevated amounts in the blood, liver damage is most likely present. Alkaline Phosphatase and GGT: Another of the liver's key functions is the production of bile, which helps digest fat. Bile flows through the liver in a system of small tubes (ducts), and is eventually stored in the gallbladder, under the liver. When bile flow is slow or blocked, blood levels of certain liver enzymes rise: Alkaline phosphatase Gamma-utamyl transpeptidase (GGT) Liver tests may check for any or all of these enzymes in the blood. Alkaline phosphatase is by far the most commonly tested of the three. If alkaline phosphatase and GGT are elevated, a problem with bile flow is most likely present. Bile flow problems can be due to a problem in the liver, the gallbladder, or the tubes connecting them. Proteins are important building blocks of all cells and tissues. Proteins are necessary for your body's growth, development, and health. Blood contains two classes of protein, albumin and globulin. Albumin proteins keep fluid from leaking out of blood vessels. Globulin proteins play an important role in your immune system. Low total protein may indicate: 1.bleeding 2.liver disorder 3.malnutrition 4.agammaglobulinemia High Protein levels 'Hyperproteinemia: May be seen in dehydration due to inadequate water intake or to excessive water loss (eg, severe vomiting, diarrhea, Addison's disease and diabetic acidosis) or as a result of increased production of proteins Low albumin levels may be caused by: 1.A poor diet (malnutrition). 2.Kidney disease. 3.Liver disease. High albumin levels may be caused by: Severe dehydration.

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NABL M(EL)T LABS NABL-M(EL)T-00728

Test Description	Value(s)	Unit(s)	Reference Range
BIOCHEMISTRY REPORT			
Smart Plus Full Body Checkup With Vitamin Screening			
Kidney Function Test (KFT)			
Blood Urea Method : Urease	20.7	mg/dL	18 - 55
Creatinine Method : Photometric	0.6	mg/dL	0.57 - 1.11
Bun * Method : Urease	**9.67**	mg/dL	9.8 - 20.1
Bun/Creatinine Ratio *	16.12		
Urea / Creatinine Ratio *	34.5		
Uric Acid Method : Uricase	3.5	mg/dL	2.6 - 6.0
Calcium Serum Method : Arsenazo III	9.4	mg/dL	8.8 - 10.0
Phosphorus Method : Photometric	4.0	mg/dL	2.3 - 4.7
Sodium Method : Potentiometric	140	mmol/L	136 - 145
Potassium Method : Potentiometric	**3.3**	mmol/L	3.5 - 5.1
Chloride Method : Potentiometric	98.7	mmol/L	98 - 107

Interpretation:

Kidney function tests is a collective term for a variety of individual tests and procedureshat can be done toevaluate how well the kidneys are functioning. Many conditions can affect the ability of the kidneys to carryout their vital functions. Somelead to a rapid (acute) decline in kidney functionothers lead to a gradual (chronic) declineinfunction. Both result in a buildup of toxic waste subst done on urine samples, as well as on blood samples. A number of symptoms may indicate a problem with your kidneys. These include : high blood pressure,blood in urine frequent urges to urinate,difficulty beginning urination,painful urination,swelling in the hands and feet due to a buildup of fluids in the body. A single symptom may not mean something serious. However, when occurring simultaneously, these symptoms suggest that your kidneys are not working properly. Kidney function tests can help determine the reason. Electrolytes (sodium,potassium,and chloride) are present in the human body and the balancing act of the electrolytes in our bodies is essential for normal function of our cells and organs. There has to be a balance.Ionized calcium this test if you have signs of kidney or parathyroid disease. The test may also be done to monitor progress and treatment of these diseases.

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NABL M(EL)T LABS NABL-M(EL)T-00728

Test Description	Value(s)	Unit(s)	Reference Range

BIOCHEMISTRY REPORT

Smart Plus Full Body Checkup With Vitamin Screening

Lipid Profile

Total Cholesterol Method : Enzymatic - Cholesterol Oxidase	152	mg/dL	<200
Triglycerides Method : Colorimetric - Lip/Glycerol Kinase	72	mg/dL	<150
HDL Cholesterol Method : Accelerator Selective Detergent	41	mg/dL	>40
Non HDL Cholesterol * Method : Calculated	111	mg/dL	<130
LDL Cholesterol * Method : Calculated	96.6	mg/dL	<100
V.L.D.L Cholesterol * Method : Calculated	14.4	mg/dL	< 30
Chol/HDL Ratio * Method : Calculated	3.71	Ratio	3.5 - 5.0
HDL/ LDL Ratio * Method : Calculated	**0.42**	Ratio	0.5 - 3.0
LDL/HDL Ratio * Method : Calculated	2.36	Ratio	-

Interpretation:

Lipid level assessments must be made following 9 to 12 hours of fasting, otherwise assay results might lead to erroneous interpretation. NCEP recommends of 3 different samples to be drawn at intervals of 1 week for harmonizing biological variables that might be encountered in single assays.

National Lipid Association Recommendations (NLA-2014)	Total Cholesterol (mg/dL)	Triglyceride (mg/dL)	LDL Cholesterol (mg/dL)	Non HDL Cholesterol (mg/dL)
Optimal	<200	<150	<100	<130
Above Optimal			100-129	130 - 159
Borderline High	200-239	150-199	130-159	160 - 189
High	>=240	200-499	160-189	190 - 219
Very High	-	>=500	>=190	>=220

HDL Cholesterol	
Low	High
<40	>=60

Risk Stratification for ASCVD (Atherosclerotic Cardiovascular Disease) by Lipid Association of India.

Risk Category	A. CAD with > 1 feature of high risk group
Extreme risk group	B. CAD with >1 feature of very high risk group of recurrent ACS (within 1 year) despite LDL-C <or = 50 mg/dl or poly vascular disease

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NABL M(EL)T LABS NABL-M(EL)T-00728

Test Description	Value(s)	Unit(s)	Reference Range

Very High Risk	1.Established ASCVD 2.Diabetes with 2 major risk factors of evidence of end organ damage 3. Familial Homozygous Hypercholesterolemia
High Risk	1. Three major ASCVD risk factors 2. Diabetes with 1 major risk factor or no evidence of end organ damage 3. CHD stage 3B or 4. 4 LDL >190 mg/dl 5. Extreme of a single risk factor 6. Coronary Artery Calcium - CAC > 300 AU 7. Lipoprotein a >/= 50 mg/dl 8. Non stenotic carotid plaque
Moderate Risk	2 major ASCVD risk factors
Low Risk	0-1 major ASCVD risk factors

Major ASCVD (Atherosclerotic cardiovascular disease) Risk Factors

1. Age >/=45 years in Males & >/= 55 years in Females	3. Current Cigarette smoking or tobacco use
2. Family history of premature ASCVD	4. High blood pressure
5. Low HDL	

Newer treatment goals and statin initiation thresholds based on the risk categories proposed by Lipid Association of India in 2020.

Risk Group	Treatment Goals		Consider Drug Therapy	
	LDL-C (mg/dl)	Non-HDL (mg/dl)	LDL-C (mg/dl)	Non-HDL (mg/dl)
Extreme Risk Group Category A	<50 (Optional goal <OR = 30)	<80 (Optional goal <OR = 60)	>OR = 50	>OR = 80
Extreme Risk Group Category B	>OR = 30	>OR = 60	> 30	> 60
Very High Risk	<50	<80	>OR = 50	>OR = 80
High Risk	<70	<100	>OR = 70	>OR = 100
Moderate Risk	<100	<130	>OR = 100	>OR = 130
Low Risk	<100	<130	>OR = 130*	>OR = 160

*** After an adequate non-pharmacological intervention for at least 3 months.**

References : Management of Dyslipidaemia for the Prevention of Stroke : Clinical practice Recommendations from the Lipid Association of India. Current Vascular Pharmacology,2022,20,134-155.

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Test Description	Value(s)	Unit(s)	Reference Range

BIOCHEMISTRY REPORT

Smart Plus Full Body Checkup With Vitamin Screening

Vitamin B12 / Cyanocobalamin

Vitamin - B12 Method : ECLIA	**82.6**	pg/mL	197 - 771

Interpretation:

Low Values are a sign of a vitamin B12 deficiency. People with this deficiency are likely to have or develop symptoms.

Causes of vitamin B12 deficiency include:Not enough vitamin B12 in diet (rare except with a strict vegetarian diet), Diseases that cause malabsorption (for example, celiac disease and Crohn's disease), Lack of intrinsic factor, Above normal heat production (for example, with hyperthyroidism), Pregnancy. Increased vitamin B12 levels are uncommon. Usually excess vitamin B12 is removed in the urine. Conditions that can increase B12 levels include: Liver disease (such as cirrhosis or hepatitis), Myeloproliferative disorders (for example, polycythemia vera and chronic myelocytic leukemia).

Vitamin B12: Low Levels can cause malabsorption, Lack of intrinsic factor, Above normal heat production (for example, with hyperthyroidism), Pregnancy.High Level Liver disease, Myeloproliferative disorders (for example, polycythemia vera and chronic myelocytic leukemia).

1. Out of 140 healthy indian population, 91% of Vitamin B 12 concentrations was at lower level: 59.00 pg/ml and upper level: 700.00 pg/ml

"Patients on Biotin supplement may have interference in some immunoassays. Ref: Arch Pathol Lab Med—Vol 141, November 2017. With individuals taking high dose Biotin (more than 5 mg per day) supplements, at least 8-hour wait time before blood draw is recommended."

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Test Description	Value(s)	Unit(s)	Reference Range

BIOCHEMISTRY REPORT

Smart Plus Full Body Checkup With Vitamin Screening

Vitamin D 25 Hydroxy

Vitamin D 25 - Hydroxy Method : ECLIA	**13.9**	ng/mL	Deficiency : <30 ng/mL

Interpretation:

25-Hydroxy vitamin D represents the main body reservoir and transport form. Mild to moderate deficiency is associated with Osteoporosis / Secondary Hyperparathyroidism while severe deficiency causes Rickets in children and Osteomalacia in adults. Prevalence of Vitamin D deficiency is approximately >50% specially in the elderly. This assay is useful for diagnosis of vitamin D deficiency and Hypervitaminosis D. It is also used for differential diagnosis of causes of Rickets & Osteomalacia and for monitoring Vitamin D replacement therapy.

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Sample Type	: Serum	Report Date	: Mar 23, 2024, 03:40 PM
Barcode No	: ZC077781	Report Status	: Final Report

Test Description	Value(s)	Unit(s)	Reference Range

BIOCHEMISTRY REPORT

Smart Plus Full Body Checkup With Vitamin Screening

Thyroid Profile Total

Triiodothyronine (T3) Method : ECLIA	1.43	ng/mL	0.80 - 2.00
TotalnThyroxine (T4) Method : ECLIA	8.5	µg/dL	5.1 - 14.1
Thyroid Stimulating Hormone (Ultrasensitive) Method : ECLIA	**5.6**	mIU/L	0.27 - 4.20

Interpretation:

Pregnancy	Reference ranges TSH
1 st Trimester	0.1 - 2.5
2 ed Trimester	0.2 - 3.0
3 rd Trimester	0.3 - 3.0

Primary malfunction of the thyroid gland may result in excessive (hyper) or below normal (hypo) release of T3 or T4. In addition as TSH directly affects thyroid function, malfunction of the pituitary or the hypo - thalamus influences the thyroid gland activity. Disease in any portion of the thyroid-pitutary-hypothala- mus system may influence the levels of T3 and T4 in the blood. In primary hypothyroidism, TSH levels are significantly elevated, while in secondary and tertiary hypothyroidism, TSH levels may be low. In addition, in the Euthyroid Sick Syndrome, multiple alterations in serum thyroid function test findings have been recognized in patients with a wide variety of non-thyroidal illnesses (NTI) without evidence of preexisting thyroid or hypothalami c-pitutary diseases. Thyroid Binding Globulin (TBG) concentrations remain relatively constant in healthy individuals. However, pregnancy, excess estrogen's, androgen's, antibiotic steroids and glucocorticoids are known to alter TBG levels and may cause false thyroid values for Total T3 and T4 tests.

TSH	T4	T3	INTERPRETATION
High	Normal	Normal	Mild (subclinical) hypothyroidism
High	Low	Low or normal	Hypothyroidism
Low	Normal	Normal	Mild (subclinical) hyperthyroidism
Low	High or normal	High or normal	Hyperthyroidism
Low	Low or normal	Low or normal	Nonthyroidal illness; pituitary (secondary) hypothyroidism
Normal	High	High	Thyroid hormone resistance syndrome (a mutation in the thyroid hormone receptor decreases thyroid hormone function)

(*) Parameter(s) are outside the scope of tests recognized under the NABL M(EL)T Scheme.

Dr. Meghana Polamarasetty
MD Pathology
Consultant Pathologist

LABORATORY REPORT

Redcliffe labs

Patient Name	**: Ms Ch Padma Vati**		
DOB/Age/Gender	: 61 Y/Female	Bill Date	: Mar 20, 2024, 05:13 PM
Patient ID / UHID	: 7653250/RCL6812930	Sample Collected	: Mar 23, 2024, 07:46 AM
Referred By	: Dr.	Sample Received	: Mar 23, 2024, 02:06 PM
Sample Type	: Spot Urine	Report Date	: Mar 23, 2024, 03:45 PM
Barcode No	: YA431928	Report Status	: Final Report

NABL M(EL)T LABS NABL-M(EL)T-00728

Test Description	Value(s)	Unit(s)	Reference Range

CLINICAL PATHOLOGY REPORT

Smart Plus Full Body Checkup With Vitamin Screening

Urine Routine and Microscopic Examination

Physical Examination			
Volume *	20	ml	-
Colour *	Pale yellow	-	Pale yellow
Transparency *	Clear	-	Clear
Deposit *	Absent	-	Absent
Chemical Examination			
Reaction (pH) Method : Double Indicator	6.0	-	4.5 - 8.0
Specific Gravity Method : Ion Exchange	1.010	-	1.010 - 1.030
Urine Glucose (sugar) Method : Oxidase / Peroxidase	Negative	-	Negative
Urine Protein (Albumin) Method : Acid / Base Colour Excahnge	Negative	-	Negative
Urine Ketones (Acetone) Method : Legals Test	Negative	-	Negative
Blood Method : Peroxidase Hemoglobin	Negative	-	Negative
Leucocyte esterase Method : Enzymatic Reaction	Negative	-	Negative
Bilirubin Urine Method : Coupling Reaction	Negative	-	Negative
Nitrite Method : Griless Test	Negative	-	Negative
Urobilinogen Method : Ehrlichs Test	Normal	-	Normal
Microscopic Examination			
Pus Cells (WBCs) *	2-4	/hpf	0 - 5
Epithelial Cells *	1-2	/hpf	0 - 4
Red blood Cells *	Absent	/hpf	Absent
Crystals *	Absent	-	Absent
Cast *	Absent	-	Absent
Yeast Cells *	Absent	-	Absent
Amorphous deposits *	Absent	-	Absent
Bacteria *	Absent	-	Absent
Protozoa *	Absent	-	Absent

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LABORATORY REPORT

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NABL M(EL)T LABS
NABL-M(EL)T-00728

Test Description	Value(s)	Unit(s)	Reference Range

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1. The presented findings in the Reports are intended solely for informational and interpretational purposes by the referring physician or other qualified medical professionals possessing a comprehensive understanding of reporting units, reference ranges, and technological limitations. The laboratory shall not be held liable for any interpretation or misinterpretation of the results, nor for any consequential or incidental damages arising from such interpretation.

2. It is to be presumed that the tests performed pertain to the specimen/sample attributed to the Customer's name or identification. It is presumed that the verification particulars have been cleared out by the customer or his/her representation at the point of generation of said specimen / sample. It is hereby clarified that the reports furnished are restricted solely to the given specimen only.

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4. This report shall not be deemed valid or admissible for any medico-legal purposes.

5. The Customers assume full responsibility for apprising the Company of any factors that may impact the test finding. These factors, among others, includes dietary intake, alcohol, or medication / drug(s) consumption, or fasting. This list of factors is only representative and not exhaustive.